Neurobiological Contributors to Depressive and Bipolar Disorders



Brain Regions and Circuits Associated with Depression

Has it ceased to have investigational hot regions involved in depression, but the research has produced conflicting findings. The situation has started to focus. Perlow Perdoy and his colleagues to conclude that identifying a single brain region as a contributor to depression is not possible because many brain regions are implicated in depression. Instead, they argue that it is the "dysfunctional interactions between brain regions" that are responsible for the dysfunctional affect and cognitive symptoms of depression. Rather than any one brain region, Mayberg et al., 2021, **Medial Cingulate** say it is interconnected networks of brain nuclei that communicate with each other (Sun & Peterson, 2019).

One way to identify brain regions that are implicated in depression is to identify symptoms of depressive disorders and then look for brain regions that are associated with those symptoms. Let's start with one of the most common symptoms. Greg Siegle and colleagues, noticed that those that showed atypical (or failed) momentary brain regions in such patients. Those brain areas appear to be part of a depression-related brain circuit (Hamilton & Hutton, 2018), which includes the subgenual anterior cingulate cortex, the amygdala, and the hippocampus. Let's start with the amygdala and the hippocampus. The image identifies brain structures involved in depression.Disorders illustrates the location of these brain regions. These interconnected brain regions receive and process incoming signals and communicate neural messages to other parts of the brain (Perlow, 2019).

Brain Structures Involved in Depressive Disorders



Note. The ventromedial area, the subgenual anterior cingulate cortex, the amygdala, and the hippocampus appear to be part of a depression-related brain circuit.

Brain Regions and Circuits Associated with Bipolar Disorder

Like many depressive disorders, bipolar disorder research centers around it's cognitive nature but rather it is associated with dysfunctions in a large number of interconnected areas. This has led the researchers interested in understanding bipolar disorder to focus on connectivity through larger networks of brain structures and how issues within these networks contribute to the symptoms of bipolar disorder.

There are conflicting findings about which brain structures are involved in bipolar disorder, likely owing to the variability in bipolar symptoms, such as the experiences of psychosis or non-psychotic symptoms (Bruno et al., 2021; Ellison-Wright & Bullmore, 2010). Regions implicated in bipolar disorder include the anterior cingulate cortex, orbitofrontal cortex, ventral striatum, hippocampus, amygdala, and insula.

There is evidence that hyper-functions, such as in the anterior cingulate cortex (ACC), the orbitofrontal cortex, the amygdala, and the insula, and hypo-functions, such as in the ventral striatum, the ACC, and the hippocampus, in bipolar disorders (Bora et al., 2017; Bora et al., 2016; Ellison-Wright & Bullmore, 2010). Other research has found decreased activity, such as in the anterior cingulate cortex and ventral striatum, which are involved with regulating emotion and reward, and increased activity in the amygdala, which is involved with processing fear (Phillips et al., 2008). Increased activity in brain areas such as the orbitofrontal cortex, amygdala, and insula, have also been linked to bipolar disorder.

The research evidence that bipolar disorder arises from the failure of the limbic network to sufficiently regulate the activity of the amygdala. This is consistent with findings that there's poor connectivity within this limbic network in bipolar patients. And some studies allege they do not have bipolar disorder (Rasmussen et al., 2017).

It's worth noting two points about the way the brain operates in the context of bipolar disorder. First, many of the brain regions implicated in bipolar disorder are also implicated in depression. Second, as mentioned in the context of psychosis, issues here can contribute either a bipolar patient's depression—or by looking at the activity of the brain areas affirm factor (Nauchand et al., 2017). Put another way, brain activity seems to come to be that rules when the person is depressed, but not when they're manic. Finally, as mentioned in the context of psychosis, the brain regions previously described, may be the result of the immune system causing inflammation in the brain, thereby damaging or destroying the tissue (Magnusson & Perlow, 2021).

The following figure, *Brain Circuits and Bipolar Disorder*, illustrates several differences areas that show effects of bipolar disorder, following the figure is a table that lists the structure and functional differences arising from bipolar disorder.

Brain Structures and Bipolar Disorder



Note. Regions implicated in bipolar disorder include the anterior cingulate cortex, orbitofrontal cortex, ventral striatum, hippocampus, amygdala, and insula.

Structural and Functional Differences Arising from Bipolar Disorder

Brain structure	Function	Activity Correlated with Bipolar Disorder	Size in patients with bipolar disorder
Anterior Cingulate Cortex (ACC)	Plays an important role in regulating and expressing emotions	N/A	Decreased
Orbitofrontal Cortex	Involved in integrating emotions into decision making	Increased	Decreased
Ventral Striatum	Involved in the perception of pleasure and reward-related behavior	Decreased	Decreased
Hippocampus	Involved in memory creation and emotional regulation	N/A	Decreased
Amygdala	Involved in the generation and processing of emotional stimuli, especially negative stimuli	Increased	Increased
Insula	Involved in regulating emotions and self-awareness	Increased	Decreased

In sum, there are many different biological mechanisms that are important in understanding the cause and treatments of depression and bipolar disorders. We know a lot about how two or more systems talk (study) and we're starting to know more and more about how they work together to create a mental disorder. The following table, *Summary of Biological Factors of Depressive and Bipolar Disorder*, summarizes the biological factors overall in this module.

Summary of Biological Factors of Depressive and Bipolar Disorders

Biological Contributors	Nature of Biological Factor
Neurotransmitters	Chemically powerful and very significant in how we feel or do anything
Genetics	Genes provide cells (the unique) for proteins needed for creation muscle assemblers, receptors, and others items (structures) in the nervous system.
Epigenetics	Environmental factors can activate or deactivate genes on our DNA.
Hormones	Hormones are chemical messengers used by the body to communicate and messages from one location to another.
Brain Circuits	Although researchers initially looked at individual brain structures and areas, they now talk about groups of interconnected brain areas involved in depression.

Mark as completed

Like Dislike Report an issue